

Patient Name : Mr. KAIF ALI	Specimen Drawn ON : 04/Feb/2026 10:00AM
Age/Gender : 19 YRS /M	Specimen Received ON : 04/Feb/2026 07:00PM
UHID/MR No : ADEL.0004092159	Report Date : 04/Feb/2026 07:30PM
Barcode No : D3062115	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
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COMPLETE BLOOD COUNT(CBC)23

R.B.C	4.94	Millions/cumm	4.5-5.5	Impedance variation
Haemoglobin	14.5	g/dl	13-17	Spectrophotometry
Packed Cell Volume	44.50	%	40.0-50.0	Analogical Integration
MCV	90.08	fL	80-100	
MCH	29.35	pg	27.0-32.0	Calculated
MCHC	32.58	g/dL	27.0-48.0	Calculated
RDW-CV	15.8	%	11.5-14.0	Calculated
Platelet Count	195	x1000/uL	150-450	Impedance Variation
Total WBC Count	4670	/cumm	4000-10000	Impedance Variation
TNC	4.67			
MPV	10.40	%	9.1-11.9	Calculated
PCT	0.20	%	0.18-0.39	Calculated
PDW	19.60	%	9.0-15.0	Calculated

Differential Leucocyte Count

Neutrophil	50	%	40.0-80.0	flow cytometry/manual
Lymphocyte	40	%	20.0-40.0	flow cytometry/manual
Monocytes	08	%	2-10	flow cytometry/manual
Eosinophils	02	%	01-06	Flow cytometry/manual
Basophils	00	%	0-2	Flow cytometry/manual
Absolute Neutrophils	2.34	1000/uL	2.00-7.00	
Absolute Lymphocytes	1.87	1000/μL	1.00-3.00	
Absolute Monocytes	0.37	1000/μL	0.20-1.00	
Absolute Eosinophils	0.09	1000/μL	0.02-0.50	
Neutrophil-Lymphocyte Ratio	1.25			Calculated
Lymphocyte-Monocyte Ratio	5			Calculated
Platelet-Lymphocyte Ratio	5			Calculated

This report has been validated by:



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Age/Gender : 19 YRS /M	Specimen Received ON : 04/Feb/2026 07:00PM
UHID/MR No : ADEL.0004092159	Report Date : 04/Feb/2026 07:54PM
Barcode No : D3062115	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE

DEPARTMENT OF HAEMATOLOGY				
Test Name	Result	Unit	Bio. Ref. Range	Method

MALARIAL PARASITE SPECIES IDENTIFICATION				
Malarial Parasite Identification	NEGATIVE			Leishman Staining & Microscopy




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Patient Name : Mr. KAIF ALI	Specimen Drawn ON : 04/Feb/2026 10:00AM
Age/Gender : 19 YRS /M	Specimen Received ON : 04/Feb/2026 06:59PM
UHID/MR No : ADEL.0004092159	Report Date : 04/Feb/2026 08:12PM
Barcode No : D3062116	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
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LIVER FUNCTION TEST (LFT)-EXTENDED

Sample Type : SERUM

Bilirubin Total	0.73	mg/dl	<1.1	Diazotized Sulfanilic
Bilirubin Direct	0.19	mg/dl	0-0.3	Diazotized Sulfanilic
Bilirubin Indirect	0.54	mg/dl	0.30-1.00	Calculated
SGOT (AST)	34.5	U/L	<31.0	IFCC without pyridoxal phosphate
SGPT (ALT)	29.4	U/L	<33.0	IFCC without pyridoxal phosphate
Alkaline Phosphatase (ALP)	93.6	U/L	55-149	Spectrophotometry
Gamma Glutamyl Transferase (GGT)	14.1	U/L	15-60	L-Gamma-glutamyl-3-carboxy-4-nitroanilide Substrate
Protein Total	7.5	g/dL	6.6-8.7	Biuret
Albumin (Serum)	5.09	g/dL	3.2-4.5	Bromo Cresol Green (BCG)
Globulin	2.41	g/dL	2.50-3.50	Calculated
A/G Ratio	2.11		1.5-2.5	Calculated

Interpretation:- Liver blood tests, or liver function tests, are used to detect and diagnose disease or inflammation of the liver. Elevated aminotransferase (ALT, AST) levels are measured as well as alkaline phosphatase, albumin, and bilirubin. Some diseases that cause abnormal levels of ALT and AST include hepatitis A, B, and C, cirrhosis, iron overload, and Tylenol liver damage. Medications also cause elevated liver enzymes. There are less common conditions and diseases that also cause elevated liver enzyme levels.: Liver blood tests, or liver function tests, are used to detect and diagnose disease or inflammation of the liver. Elevated aminotransferase (ALT, AST) levels are measured as well as alkaline phosphatase, albumin, and bilirubin. Some diseases that cause abnormal levels of ALT and AST include hepatitis A, B, and C, cirrhosis, iron overload, and Tylenol liver damage. Medications also cause elevated liver enzymes. There are less common conditions and diseases that also cause elevated liver enzyme levels.

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Patient Name : Mr. KAIF ALI	Specimen Drawn ON : 04/Feb/2026 10:00AM
Age/Gender : 19 YRS /M	Specimen Received ON : 04/Feb/2026 06:59PM
UHID/MR No : ADEL.0004092159	Report Date : 04/Feb/2026 08:25PM
Barcode No : D3062116	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE

DEPARTMENT OF IMMUNOASSAY

Test Name	Result	Unit	Bio. Ref. Range	Method
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THYROID PROFILE

Sample Type : SERUM

Triiodothyronine Total (T3)	1.05	ng/mL	0.80-2.10	Chemiluminescence Immunoassay (CLIA)
Thyroxine Total (T4)	7.33	ug/dL	4.6-10.5	Chemiluminescence Immunoassay (CLIA)
TSH (4th Generation)	2.700	uIU/mL	0.70-6.40	Chemiluminescence Immunoassay (CLIA)

PREGNANCY	REFERENCE RANGE for TSH IN uIU/mL (As per American Thyroid Association.)
1st Trimester	0.10-2.50 uIU/mL
2nd Trimester	0.20-3.00 uIU/mL
3rd Trimester	0.30-3.00 uIU/mL

INTERPRETATION-

1. Primary hyperthyroidism is accompanied by elevated serum T3 & T4 values along with depressed TSH level.
2. Primary hypothyroidism is accompanied by depressed serum T3 and T4 values & elevated serum TSH levels.
3. Normal T4 levels accompanied by high T3 levels and low TSH are seen in patients with T3 thyrotoxicosis.
4. Normal or low T3 & high T4 levels indicate T4 thyrotoxicosis (problem is conversion of T4 to T3)
5. Normal T3 & T4 along with low TSH indicate mild / subclinical HYPERTHYROIDISM .
6. Normal T3 & low T4 along with high TSH is seen in HYPOTHYROIDISM .
7. Normal T3 & T4 levels with high TSH indicate Mild / Subclinical HYPOTHYROIDISM .
8. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness , malnutrition , renal failure and during therapy with drugs like propranolol.
9. Although elevated TSH levels are nearly always indicative of primary hypothyroidism . rarely they can result from TSH secreting pituitary tumours (secondary hyperthyroidism)

TSH IS DONE BY ULTRASENSITIVE 4th GENERATION CHEMIFLEX ASSAY

COMMENTS:

Assay results should be interpreted in context to the clinical condition and associated results of other investigations. Previous treatment with corticosteroid therapy may result in lower TSH levels while thyroid hormone levels are normal. Note:

TSH levels may fluctuate based on few factors such as pregnancy, illness and age. Also, time of sample collection, technologies used to analyze the test, usage of certain drugs. Diet may have impact on TSH levels. TSH may show around 50% variation even when done at different times of day due to its association with circadian rhythm.

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Laboratory Test Report

Patient Name : Mr. KAIF ALI	Specimen Drawn ON : 04/Feb/2026 10:00AM
Age/Gender : 19 YRS /M	Specimen Received ON : 04/Feb/2026 06:59PM
UHID/MR No : ADEL.0004092159	Report Date : 04/Feb/2026 10:43PM
Barcode No : D3062116	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE

DEPARTMENT OF SEROLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
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HIV I & II Antibody	0.01	AU/mL	0-1	CLIA
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COMMENTS: Non reactive results indicate that antibodies to HIV I/II have not been detected in the sample. This implies either non exposure to HIV I/II or that the individual might be in the 'Window Period', that is, prior to the development of detectable level of Antibodies. Therefore, a non-reactive result does not exclude the possibility of a persisting HIV infection. Reactive samples must be confirmed by using HIV Western Blot as some degree of cross reactivity for HIV antibodies has been noted with certain other naturally occurring antibodies and also antibodies formed in response to other bacterial and viral infections, and in certain autoimmune disorders. Post test counseling will be done by the concerned referring doctor. The sensitivity and specificity of this test has been determined by National HIV Reference Centers of Govt.of India and WHO collaborating center, using various other test panels.

This report has been validated by:



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Patient Name : Mr. KAIF ALI	Specimen Drawn ON : 04/Feb/2026 10:00AM
Age/Gender : 19 YRS /M	Specimen Received ON : 04/Feb/2026 06:59PM
UHID/MR No : ADEL.0004092159	Report Date : 05/Feb/2026 12:52AM
Barcode No : D3062116	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE

DEPARTMENT OF SEROLOGY				
Test Name	Result	Unit	Bio. Ref. Range	Method

WIDAL (SLIDE AGGLUTINATION)

Sample Type : SERUM

S. Typhi Antigen (O)	<1:80		<1:80 Negative	Agglutination
S. Typhi Antigen (H)	<1:80		<1:80 Negative	Agglutination
S. Paratyphi Antigen (AH)	<1:80		<1:80 Negative	Agglutination
S. Paratyphi Antigen (BH)	<1:80		<1:80 Negative	Agglutination

COMMENT:

Significant titer: For 'O' Antigen- agglutination titer of >1/80 is considered significant.

For 'H' Antigen- agglutination titer of >1/80 is considered significant.

In Salmonella Typhi infection- Antibodies to TO & TH antigens are raised.

In Salmonella Paratyphi A infection- Antibodies to TO & AH antigens are raised.

In Salmonella Paratyphi B infection- Antibodies to TO & BH antigens are raised.

NOTE:

Widal test is indicated in 2-3 weeks of illness. It is a slide agglutination test that detects antibodies in the patient's serum against antigens of S. Typhi & S. Paratyphi. Reactive results indicate recent or ongoing infection by Salmonella species & the diagnosis should be confirmed by Blood culture which is the gold standard test. A single positive test result has less significance, therefore repeat testing is recommended after 1 week to demonstrate rise in titer which is considered as definitive evidence of infection.

False negative results may be seen due to following reasons: very early stage (1st week) of illness, antibiotic therapy or prozone phenomena.

False positive results may be seen due to following reasons: prior immunization or unrelated infections.

*** End Of Report ***

This report has been validated by:


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Age/Gender : 19 Y 0 M 0 D /M	Specimen Received ON : 04/Feb/2026 06:59PM
UHID/MR No : ADEL.0004092159	Report Date : 10/Feb/2026 10:58AM
Barcode No : D3062117	Client Code : DL2838
Ref Doctor : Dr.SELF	Visit ID : MDEL4095247
Ref Customer : SELF	Client Name : LIFE CARE DIAGNOSTIC CENTRE.

DEPARTMENT OF MICROBIOLOGY

TEST NAME : CULTURE BLOOD (AEROBIC)		
SPECIMEN TYPE : 6ML.(5ML MINIMUM) BLOOD IN GREEN TOP HEPARIN VEIL		
ORGANISM ISOLATED	NO GROWTH SEEN.	
PRELIMINARY REPORT	CULTURE STERILE AFTER 48 HOURS OF INCUBATION AT 37°C	
FINAL REPORT	CULTURE STERILE AFTER 7 DAYS OF INCUBATION AT 37°C	IU/ML

Comment:

Method: Conventional culture and sensitivity.

Kirby Bauer Disc diffusion Method as per CLSI Guidelines for Reporting Sensitivity.

Note: - 1st Report for Blood Culture is given after 48 hrs & final report is done after 7 days.
If Positive shall be reported earlier.

***** End Of Report *****

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